CLINICAL PSYCHOLOGY · DE-ADDICTION

Motivational Enhancement Therapy

Book Two — Session-by-Session Guide & Worksheet Pack

12-Session Program · Techniques per Session · 24 Ready-to-Use Worksheets

Exactly what to do from the first session to the last, which technique to use in which session, and a complete pack of long, comprehensive, printable worksheets — including MET short-term and long-term goal worksheets. Companion to Book One (Theory & Techniques).

Dango_Psych — Tejas Dangodra

M.Phil. Clinical Psychology

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Part A · How to Use This Guide

This book turns the theory and techniques of Book One into action. Each session lists the **techniques used** (with their Book-One codes, e.g. A3 = reflective listening, C2 = rulers) so you can see exactly which skill to apply when.

> **Technique code key (full detail in Book One §9). A** = OARS (A1 open Qs, A2 affirmations, A3 reflections, A4 summaries); **B** = reflections (B1 simple…B5 feeling); **C** = evoking (C1 evocative Qs, C2 rulers, C3 extremes, C4 look back/forward, C5 values, C6 elaborate, C7 key question, C8 bouquet); **D** = sustain talk/discord (D1 roll, D2 autonomy, D3 reframe, D4 agree-with-twist, D5 shift, D6 come alongside, D7 repair); **E** = info (E1 E-P-E, E2 feedback, E3 permission, E4 menu); **F** = planning/relapse (F1 decisional balance … F13 boosters).

Assessment before MET

A structured assessment precedes MET; its results feed the Session-2 feedback. Cover: present use (type/amount/frequency/**route**/duration), last use & cravings, triggers, consequences (physical/psychological/family/work/legal), previous attempts, withdrawal & overdose risk, patient's own understanding, readiness/importance/confidence, strengths & supports, barriers, and goals/preferences. Instruments: WHO ASSIST, AUDIT, Fagerström, timeline follow-back, SOCRATES.

> **Flag immediately** for medical/psychiatric care: suicidal ideation, overdose risk, severe alcohol/benzodiazepine withdrawal risk (seizure/DT history), pregnancy, acute illness. MET complements — never replaces — medical treatment.

The 12-session program at a glance

Phase	Sessions	MI process	Primary tasks	Main techniques
I · Engagement & Feedback	1–3	Engaging + Focusing	Rapport, assessment feedback, build discrepancy, map readiness	A1–A4, B1–B3, E1–E3, C2, C5
II · Evoking Motivation	4–6	Evoking	Decisional balance, values, resolve ambivalence, build confidence	F1, C1–C8, D1–D7, B3–B4
III · Planning & Action	7–9	Planning	Change plan, triggers & coping, support, pharmacotherapy	C7, F2–F8, F11–F12, E4
IV · Maintenance	10–12 (+boosters)	Planning / re-engaging	Review change, relapse prevention, continuing care, discharge	A2, F9–F10, F13, C8

SUPPLEMENTARY structure The published MET is brief (~4 sessions); this 12-session version spreads the same evidence-based tasks over more time for complex/opioid/adolescent cases. Adapt the number to the patient — the *sequence of tasks* matters more than the exact count.

Phase I · Engagement & Assessment Feedback (S1–3)

Session 1 · Engagement & Rapport

Session 1

Techniques used: A1 open questions · A3 reflective listening · A2 affirmations · A4 summaries · D2 autonomy · B2 complex reflection | **Worksheet:** #1

Objectives: establish a trusting, collaborative alliance; orient the patient to MET; understand their world; instil hope. **Rationale:** engagement predicts retention — nothing works without it. **Preparation:** private space; review referral; set a non-rushed tone; prepare consent/confidentiality points.

Step-by-step

1. Warm welcome; introduce MET collaboratively.
2. Explain confidentiality & its limits.
3. Open-ended invitation to tell their story (A1).
4. Listen with OARS; reflect far more than you ask (A3).
5. Elicit hopes/concerns; affirm attending (A2, D2).
6. Agree a loose agenda for the sessions ahead.

T: "Thanks for coming in. Before anything else — what's been going on in your life lately, in your own words?"
P: "Honestly, I'm only here because my wife pushed me."
T (D2 + B2): "You came partly for her — and whatever we do here is still your choice. While you're here, what would make this time worth it for *you*?"
P: "I suppose I'm tired of the arguments."

Common mistakes: rushing to forms before rapport; siding with family against the patient; correcting denial; filling silences. **Homework:** notice one thing about use they'd want different. **Documentation:** engagement quality, stated reason for attending, early change/sustain talk, agenda, risk screen. **Move on when:** a working alliance exists & the patient will continue.

Session 2 · Personalised Assessment Feedback

Session 2

Techniques used: E3 ask permission · E2 personalised feedback · E1 Elicit-Provide-Elicit · B3 double-sided reflection · A4 summary · C2 rulers | **Worksheets:** #2, #4, #5, #6

Objectives: deliver structured, personalised assessment feedback in MI style (the "enhancement"); let the patient draw their own conclusions. **Rationale:** objective feedback given without argument

raises motivation while protecting autonomy. **Preparation:** a one-page feedback sheet (levels vs norms, risk markers, consequences); rulers ready.

Step-by-step (E-P-E)

1. Ask permission to share results (E3).
2. Elicit what the patient already knows/expects.
3. Provide one finding at a time, neutrally; pause (E2).
4. Reflect their reaction; avoid defending the data (B3).
5. Summarise; ask "What do you make of all this?" (A4).

T (E2): "Your reported use is in the high-risk range compared with most men your age. How does that land?"
P: "I don't use more than my friends."
T (B3): "Among your circle this feels normal — and seeing it in the high-risk range is a surprise."
P: "The liver result did scare me a bit."

Mistakes: lecturing from the sheet; dumping all findings at once; pushing a goal too early. **Homework:** reflect on which feedback felt most relevant. **Move on when:** the patient engages with the feedback and voices some concern.

Session 3 · Developing Discrepancy & Mapping Readiness

Session 3

Techniques used: C2 rulers · C5 values/discrepancy · A3 reflection · C8 summarise change talk · B2 complex reflection | **Worksheets:** #4, #5, #6, #8

Objectives: develop discrepancy between use and the patient's goals/values; map importance, confidence & readiness. **Rationale:** discrepancy felt in the patient's own terms is the motor of change. **Preparation:** ruler & "why this number" prompts.

Step-by-step

1. Review the week & any homework.
2. Importance ruler "why that number?" "why not lower?" (C2).
3. Confidence ruler the same way (C2).
4. Reflect/summarise all change talk (C8).
5. Gently hold up the gap: current use vs stated hopes (C5).

T: "On 0–10, how important is changing right now?" **P:** "Maybe a 6." **T:** "Why a 6 and not a 3?" **P: "My health scares me and my wife's at her limit."** **T:** "What would move it from a 6 to an 8?"

Mistakes: creating discrepancy by lecturing; treating ruler numbers as diagnoses. **Move on to Phase II when:** the patient voices meaningful discrepancy and some desire/reason to change.

Phase II · Evoking Motivation (S4–6)

Session 4 · Exploring Ambivalence & Decisional Balance

Session 4

Techniques used: F1 decisional balance · C1 evocative questions · B3 double-sided reflection · D2 autonomy · C7 key question | **Worksheets:** #7, #11

Objectives: explore the real benefits *and* costs of using and of changing; let the balance tip in the patient's own words. **Rationale:** honouring perceived benefits first lowers defensiveness. **Preparation:** decisional-balance worksheet; resist arguing the pro-change side.

Step-by-step

1. Review week.
2. Explore “good things / things you'd miss” about use.
3. Explore “less-good things” about use.
4. Explore benefits & worries about changing.
5. Summarise all four quadrants; ask the key question (C7).

T: “What are the good things about using — the things you'd miss?” **P:** “It calms me after work.” **T:** “And the not-so-good side?” **P: “I'm broke, and we fight about it.”** **T (C7):** “Given both sides, where does that leave you?”

Mistakes: listing only cons of use; taking the pro-change side (patient then argues for use). **Move on when:** costs of using and benefits of change feature prominently in the patient's speech.

Session 5 · Personal Values & Developing Discrepancy

Session 5

Techniques used: C5 values/discrepancy · C4 looking back/forward · A3 reflection · C6 elaborate · C8 bouquet | **Worksheets:** #8, #9

Objectives: connect change to core values & life goals; deepen discrepancy; strengthen intrinsic motivation. **Rationale:** change rooted in values is more durable than change driven by fear or pressure. **Preparation:** values worksheet; allow silences to work.

Step-by-step

1. Elicit top values (“What matters most to you?”).
2. Explore how the patient wants to be seen/remembered (C4).
3. Explore how use fits — or clashes — with those values (C5).

4. Reflect the discrepancy; let *them* name it (A3).

> **T:** "Being a good father is everything to you. How does using fit with the father you want to be?" **P: "It doesn't. That's not who I want to be."** **T:** "There's a real gap between how you're living and who you want to be — and you want to close it."

Mistakes: imposing your values/moralising; rushing past emotion. **Move on when:** the patient articulates a values-based reason for change.

Session 6 · Building Confidence & Self-Efficacy

Session 6

Techniques used: C2 confidence ruler · F10 supporting self-efficacy · A2 affirmations · C4 looking back · C6 elaborate | **Worksheets:** #5, #9, #12

Objectives: raise confidence that change is possible; address the "I want to but I can't" gap; surface strengths, supports & past successes. **Rationale:** importance and confidence are independent — high importance + low confidence needs efficacy-building, not more reasons. **Preparation:** confidence ruler; strengths worksheet.

Step-by-step

1. Confidence ruler "why that number?" "why not lower?" (C2).
2. Review past successes (any domain) & how they were achieved (C4, F10).
3. Elicit strengths, skills & supports.
4. Ask "What would raise your confidence by one point?".
5. Affirm; summarise ability change talk (A2).

> **T:** "Confidence 0–10 that you could change if you decided?" **P:** "About a 3." **T:** "Why a 3 and not a 0?" **P: "I did stop for three weeks once."** **T (A2+F10):** "You've already proven you can — that's a strength. What would nudge the 3 toward a 5?"

Mistakes: hollow reassurance; jumping to planning before confidence is adequate. **Move on to Phase III when:** importance *and* confidence are both reasonably present and the patient signals readiness.

Phase III · Planning & Action (S7–9)

Session 7 · Negotiating the Change Plan

Session 7

Techniques used: C7 key question · F2 change-plan negotiation · F3 SMART goals · E4 menu of options · C8 bouquet | **Worksheets:** #14, #15, #22, #23

Objectives: confirm readiness & the chosen goal; co-create a concrete, patient-authored change plan. **Rationale:** a plan the patient writes carries commitment; a prescribed plan does not. **Preparation:** change-plan & goal worksheets; know local treatment options.

Step-by-step

1. Recap motivation ("Last time you said… where are you now?").
2. Clarify the goal in the patient's words (abstinence or reduction).
3. Offer a menu of options; the patient chooses (E4).
4. Draft steps: who/what/when (F2, F3).
5. Elicit commitment language; summarise the plan back (C8).

T (C7): "Given all we've discussed, what do you think you'll do?" **P: "I want to stop the smack. I'll ask the doctor about the medicine."** **T:** "What's the very first step this week — the appointment, telling your wife, or changing your evenings?"

Mistakes: writing the plan *for* the patient; over-ambitious goals. **Move on when:** a written, patient-owned plan with a concrete first step exists.

Session 8 · Triggers, High-Risk Situations & Coping

Session 8

Techniques used: F4 functional analysis · F5 if-then plans · F6 urge-surfing · F7 play-the-tape · F8 refusal rehearsal | **Worksheets:** #10, #12, #13, #11

Objectives: identify triggers & high-risk situations; build a personalised coping toolkit; begin craving management. **Rationale:** most lapses occur in predictable high-risk situations; planning for them protects the change. **Preparation:** triggers & coping worksheets.

Step-by-step

1. Map triggers (people/places/feelings/times) using a recent episode (F4).
2. Rank the top high-risk situations.
3. Brainstorm coping options per trigger; the patient selects.

4. Rehearse one (refusal, delay, distraction, call support) (F8).
5. Introduce craving skills: urge-surfing & play-the-tape (F6, F7).

> **T:** "When a craving hits in the evening, what tends to happen?" **P:** "I message my old group." **T (F5):** "That message is the risky moment. What could you do in the 10 minutes before you'd normally send it?" **P: "Call my brother or go for a walk."**

Mistakes: generic advice not tied to real triggers; skipping rehearsal. **Move on when:** the patient has a concrete coping plan for their main triggers.

Session 9 · Social Support & Pharmacotherapy Coordination

Session 9

Techniques used: F11 social support · F12 pharmacotherapy coordination · E1 E-P-E · E4 menu · D2 autonomy | **Worksheets:** #12, #16

Objectives: strengthen the support network; address treatment adherence; coordinate medical & pharmacological care; involve family where helpful. **Rationale:** social support and appropriate medication (MOUD, naltrexone, NRT) markedly improve outcomes. **Preparation:** referral pathways; optional family session.

Step-by-step

1. Map who supports the patient & how (F11).
2. Discuss medication neutrally via E-P-E; coordinate with the doctor (F12, E1).
3. Address adherence barriers — cost, transport, stigma, side-effects.
4. If appropriate, plan a supportive family session.
5. For opioids: naloxone counselling & overdose-prevention plan.

> **T (E1):** "What do you know about the medicine for opioid cravings? … May I add a little? … Buprenorphine can ease withdrawal so the counselling has room to work. What do you make of that?" **P: "If it helps the withdrawal, I'd try it."**

Mistakes: pushing medication rather than offering via E-P-E; ignoring access barriers. **Move on to Phase IV when:** supports & treatment are in place and the patient is enacting the plan.

Phase IV · Maintenance & Consolidation (S10–12 + boosters)

Session 10 · Reviewing Early Change & Reinforcing Self-Efficacy

Session 10

Techniques used: A2 affirmations · F10 self-efficacy · A3 reflection · C6 elaborate | **Worksheet:** #17

Objectives: review progress & setbacks without judgement; reinforce self-efficacy; adjust the plan. **Rationale:** early change is fragile; affirming genuine gains builds durable confidence.

Step-by-step

1. "What's gone well? What's been hard?"
2. Affirm concrete successes; attribute to effort (A2).
3. Explore any slips with curiosity, not blame.
4. Adjust steps/goals as needed.

T: "Two weeks with just one slip — real progress. What helped most?" **P: "Avoiding the market lane and calling my brother."** **T (A2):** "You used your plan at the exact right moment — a skill you can repeat."

Mistakes: focusing on slips over successes; generic praise.

Session 11 · Relapse Prevention & Learning from Lapses

Session 11

Techniques used: F9 lapse-vs-relapse reframing · F5 if-then plans · F8 rehearsal · A2 affirmations | **Worksheets:** #18, #19

Objectives: strengthen relapse-prevention; distinguish a lapse from a relapse; create an emergency "if-then" plan; de-shame slips. **Rationale:** treating a lapse as total failure (abstinence-violation effect) is a key relapse driver.

Step-by-step

1. Identify remaining high-risk situations & early warning signs.
2. Build "if-then" coping plans for each (F5).
3. Normalise lapses; separate lapse from relapse (F9).
4. Create an emergency plan (who to call, what to do).
5. Rehearse a difficult scenario (F8).

T (F9): “You used once last week and then stopped yourself. That slip doesn't erase three good weeks — what helped you get back on track?” **P: “I called my brother instead of my old friends.”**

Mistakes: treating a lapse as failure; vague unrehearsed plans.

Session 12 · Consolidation, Continuing Care & Discharge

Session 12

Techniques used: C8 bouquet/summary · A2 affirmations · F13 booster planning · C5 values | **Worksheets:** #20, #21, #22, #23

Objectives: consolidate gains; review the whole journey; arrange continuing care & follow-up; close respectfully & hopefully. **Rationale:** structured continuing care protects against drop-off after active treatment ends.

Step-by-step

1. Review progress from Session 1 to now; highlight growth (C8).
2. Surface remaining ambivalence; revisit the plan.
3. Agree a continuing-care pathway (clinic, group, follow-up dates, family role).
4. Affirm agency; leave the door open (A2).
5. Set booster/follow-up schedule (F13).

T: “Twelve weeks ago you were unsure change was even possible. Look at what you've built. What are you proudest of?” **P: “That my daughter trusts me again.”**

Booster sessions (F13): brief contacts at **1, 3, 6 & 12 months** — review progress re-rate rulers re-affirm change talk update plan reinforce continuing care. Especially valuable for opioids (relapse-overdose risk) and adolescents.

Substance-Specific Notes · Adolescents · Challenges

Substance-specific adaptation (method constant, content differs)

Substance	Triggers	Perceived benefits	Key consequences	Goal & barriers
Opioids/smack	Withdrawal fear, peers, pain	Relief of pain/withdrawal, euphoria	Overdose, injecting infections, debt	Often abstinence + MOUD; barriers: withdrawal, access, stigma
Alcohol	Social, stress, routine	Relaxation, social ease, sleep	Liver disease, accidents, dependence	Abstinence/reduction; barriers: norms, withdrawal risk (seizures/DTs)
Tobacco	After meals, tea, stress	Alertness, stress relief	Heart/lung disease, cost	Reduction quit + NRT; barriers: strong habit
Cannabis	Boredom, peers, sleep	Relaxation, sleep	Amotivation, anxiety/psychosis risk	Often reduction; barriers: “harmless” belief
Stimulants	Parties, work pressure	Energy, confidence	Crash, cardiac risk, bingeing	Abstinence; barriers: binge-cycle cravings
Polysubstance	Overlapping cues	Varies; self-medication	Compounded risk (opioids+sedatives)	Prioritise highest-risk; needs medical lead

Opioids: tolerance falls after any abstinence relapse carries high overdose risk; provide naloxone & facilitate MOUD. **Alcohol:** seizure/DT history needs medically-supervised withdrawal — never abrupt cessation without medical cover.

Adolescents & young adults ADOLESCENT

Same spirit & skills, adapted delivery: autonomy first and loudest; present-focused motivators (money, image, sport, school) over distant health risks; longer engagement, shorter sessions; confidentiality balanced with safeguarding and age-appropriate family involvement.

Common clinical challenges (reflect affirm autonomy evoke)

Patient says…	MET-consistent response
“I don't have a problem.”	“From where you stand, this isn't a problem. What made others feel it might be?”
“I can stop whenever I want.”	“You feel in control — and it's your choice. If you did test that, what might you try first?”
“My family forced me to come.”	“You're here for them. While you're here, what would make it worth it for *you*?”
“It's the only thing that helps me.”	“Right now it's your main way to cope. What else has ever given you a bit of that relief?”
“I tried before and failed.”	“You did stay clean three weeks — what worked, even briefly?”
A lapse after the plan.	“You slipped once then stopped yourself — that recovery matters. What can we learn?”

Part B · The Worksheet Pack

24 comprehensive, printable worksheets. Each starts on its own page with a clear **purpose**, step-by-step **"How to fill this in"** guidance, a worked example, generous space to write, and a clinician review note. Select worksheets by the patient's readiness — do not use all of them with every patient.

> **Choosing worksheets by phase:** Phase I–II (awareness/ambivalence): #1–#9. Phase III (planning/action): #10–#16, #22. Phase IV (maintenance): #17–#21, #23. The clinician session note (#24) is used after every session.

Worksheet 1 · My Substance-Use Story

Purpose: to build an honest, shared, non-judgemental picture of your substance use so you and your therapist understand the full situation. **Best used in:** assessment / Session 1.

How to fill this in:

1. Work through each box slowly — there are no right or wrong answers.
2. Write what you can; “not sure” or “don't remember” is completely fine.
3. Think about a *typical* week, not your best or worst day.
4. Include tobacco, alcohol and any medicines you use to feel different — not just the “main” drug.

Worked example: Main substance: heroin (“smack”), inhaled. Started at 25, using about 4 years. Typical use: 0.4–0.6 g/day, mostly evenings. Also: 20 cigarettes/day; occasional alcohol at weekends. Last use: this morning. Longest period not using: 3 weeks, after a detox last year. How I take it: foil (chasing), never injected.

Over to you:

Main substance & how you take it (route):

Age you first used · how many years you've been using:

Typical amount & how often now (a usual week):

Other substances you use (tobacco, alcohol, medicines, others):

When you last used · the longest you've ever gone without:

Anything about your use you'd like your therapist to understand:

Clinician review: reflect patterns and affirm honesty; note route, injecting and overdose markers for safety. If use seems under-reported, don't challenge — ask “roughly, on a heavy day?” and revisit once trust grows.

Worksheet 2 · How Substance Use Affects My Life

Purpose: to notice, across every area of life, how substance use is affecting you — the good and the costly. This often surfaces reasons to change in your own words. **Best used in:** Session 2.

How to fill this in:

1. Go area by area. For each, write any effect you've noticed — even small ones.
2. Be honest about helpful effects too; this isn't about judging yourself.
3. If you can't think of an effect in an area, imagine what someone close to you might write there.

Worked example (one row): Family/relationships — “My wife and I argue about money most nights; my daughter asked why I'm always tired.”

Area of life	How my use affects it (good & not-so-good)
Health & body	
Family & relationships	
Work / studies / money	
Mood & how I feel about myself	
Legal / safety / other	

Looking at everything above, the one effect that bothers me most is:

Clinician review: reflect without piling on — let the list speak. Highlight any effect the patient flags as most bothersome (strong change talk). If “no effects”, explore a close other's likely view.

Worksheet 3 · My Reasons for Considering Change

Purpose: to gather *your own* reasons for thinking about change — the motivators that will matter most to you. **Best used in:** Sessions 1–3.

How to fill this in:

1. List anything, big or small, that makes you wonder about changing — nothing is too minor.
2. Write in your own words; these are *your* reasons, not anyone else's.
3. Then rank your top three by circling or numbering them.

Worked example: "Money is always short · fights with my wife · my daughter noticed · my chest hurts in the mornings · I'm tired of hiding it · I want to feel in control again." Top three: money, daughter, feeling in control.

My reasons for considering change (write as many as you can):

My top three reasons (the ones that matter most to me):

Clinician review: reflect & amplify each reason (this is change talk); link reasons to values (Worksheet 8). If none offered, ask "Even a small nagging reason counts — what first made you willing to talk?"

Worksheet 4 · Importance Ruler

Purpose: to measure, in your own view, how **important** it feels to change your substance use right now.
Best used in: Sessions 2–3 and whenever motivation is being reviewed.

How to fill this in:

1. Read the question, then circle the number that fits you today (0 = not important at all, 10 = the most important thing in my life right now).
2. Answer the two questions below the ruler — these are the important part.
3. There is no "correct" number; it can change week to week.

"How important is it to you to change your substance use right now?"

0	1	2	3	4	5	6	7	8	9	10

Worked example: circled 6. "Why a 6 and not a 3? Because my health scares me and my wife is at her limit." "What would move it to an 8? If the money problems got worse, or if my daughter said something again."

Why did you choose that number, and not a lower one?

What would move your number one or two points higher?

Clinician review: the number is a conversation starter, never a diagnosis. "Why not lower?" pulls change talk; avoid "why not higher?". If importance is high but confidence low, move to confidence-building (Worksheet 5, 9).

Worksheet 5 · Confidence Ruler

Purpose: to measure how **confident** you feel that you *could* change if you decided to. Importance and confidence are different — you can badly want change yet doubt you can do it. **Best used in:** Session 6 and throughout.

How to fill this in:

1. Circle the number for how confident you feel today (0 = not at all confident, 10 = completely confident).
2. Answer the questions below — especially what would raise your confidence.
3. Low confidence is normal and workable; it tells us where to build skills and support.

"If you decided to change, how confident are you that you could?"

0	1	2	3	4	5	6	7	8	9	10

Worked example: circled 3. "Why a 3 and not a 0? Because I did stop for three weeks once." "What would raise it to a 5? Medicine for the withdrawal, and my brother checking in on me."

Why that number, and not a lower one?

What would raise your confidence by one or two points?

A time I succeeded at something hard before (and how I did it):

Clinician review: high importance + low confidence needs skills/support, not more reasons. Mine past successes for self-efficacy; translate "what would raise it" into concrete plan steps (Worksheet 14).

Worksheet 6 · Why This Number? / Why Not Lower?

Purpose: to draw out, in detail, the thinking behind your ruler scores — because the reasons you give are exactly the motivation that drives change. **Best used in:** straight after the rulers (Worksheets 4 & 5).

How to fill this in:

1. Write your importance number and your confidence number at the top.
2. For each, explain why it isn't *lower* — list every reason you can.
3. Then write what a higher number would look like in real life.

Worked example: Importance 6 — “A 3 would mean I don't care, but I do, especially about my daughter and my health.” A higher number would look like “me actually booking the clinic and telling my wife”.

My importance number: ___ / 10	**My confidence number: ___ / 10**
Why not lower? (all my reasons)	Why not lower? (all my reasons)

What a higher number would look like in my real life:

Clinician review: everything written here is change talk — reflect and summarise it back (“the bouquet”). Use “what would a higher number look like” to bridge toward the change plan.

Worksheet 7 · Decisional Balance (Four-Quadrant)

Purpose: to weigh the full picture — the pros and cons of *continuing* use and of *changing* — so you can see where the balance really sits for you. **Best used in:** Session 4 (contemplation).

How to fill this in:

1. Start with the top-left box (good things about using) — it's important to be honest about what use does *for* you.
2. Then fill the costs of using, the benefits of changing, and the worries about changing.
3. Don't count or score the boxes — read them over and notice what stands out.
4. Write your overall sense at the bottom.

Worked example: Pros of using: “relaxes me, fits in with friends.” Cons of using: “debt, arguments, poor sleep, shame.” Pros of changing: “save money, peace at home, better health, pride.” Worries about changing: “losing my friends, boredom, facing stress without it.”

	Continuing to use	Changing
Good things / Pros		
Costs / Cons		

When I read all four boxes together, my overall sense is:

Clinician review: validate the benefits of use first (lowers defensiveness), then reflect costs. Summarise all four quadrants and ask the key question “Where does this leave you?”. Never argue if the balance currently favours use — stay curious.

Worksheet 8 · My Personal Values & Goals

Purpose: to connect any change to what matters most to you as a person. Change rooted in your own values lasts far longer than change driven by pressure or fear. **Best used in:** Session 5.

How to fill this in:

1. List your most important values — the things that make life meaningful (e.g., family, honesty, faith, health, respect, freedom).
2. For each, write one line on how you'd like to be living it.
3. Then honestly note how your substance use fits — or clashes — with each value.

Worked example: Value: “being a good father” want to be present and trusted “using clashes badly — I'm distant and my daughter noticed.” Value: “financial security” “using has put us in debt.”

What I value most	How I want to live it	How use fits or clashes with it

The gap between how I'm living now and the person I want to be is:

Clinician review: this develops discrepancy — hold up the gap gently and let the patient name it. If use *supports* a value (e.g., belonging), explore other ways to meet that need.

Worksheet 9 · My Strengths & Past Successes

Purpose: to build your confidence by remembering the strengths and successes you already have. If you've overcome hard things before, you can again. **Best used in:** Session 6 / whenever confidence is low.

How to fill this in:

1. List personal strengths — qualities, skills, habits (e.g., determined, caring, good with my hands, loyal).
2. List things you've succeeded at in *any* part of life — work, parenting, sport, past quit attempts.
3. For your biggest success, write *how* you did it — those are skills you can reuse.
4. List people who believe in you.

Worked example: Strengths: “stubborn in a good way, reliable, love my family.” Successes: “held a job 6 years; quit smoking for a year once; stopped smack for 3 weeks.” How: “kept busy, avoided old friends, leaned on my brother.” Believers: “my wife and brother.”

My personal strengths:

Things I've succeeded at before (any area of life):

My biggest success and exactly how I achieved it:

People who believe in me:

Clinician review: affirm specifically and link strengths to the change goal. If the patient offers none, ask about non-addiction successes. Carry the “how I did it” skills into the change plan.

Worksheet 10 · My Triggers & High-Risk Situations

Purpose: to spot the situations, feelings and people that make using most likely — because once you can see a trigger coming, you can plan for it. **Best used in:** Session 8.

How to fill this in:

1. Think of the last few times you used. What was happening just before each time?
2. Fill each column with your real triggers — be specific (not “stress” but “stress after a shift when I have cash”).
3. Then rank your top three riskiest situations at the bottom.

Worked example: People: old using friends, a particular dealer. Places: the lane behind the market. Feelings: boredom, stress, loneliness. Times/situations: evenings after work, payday, weekends. Top three: payday, evening boredom, meeting old friends.

People	Places	Feelings	Times / situations

My top three highest-risk situations:

Clinician review: link each high-risk situation to a coping option (Worksheet 12). If triggers are vague, walk through the most recent episode step by step (functional analysis).

Worksheet 11 · My Barriers to Change

Purpose: to name the obstacles that could get in the way, so they can be planned for rather than derail you later. **Best used in:** Sessions 4 & 8.

How to fill this in:

1. List everything that might make change hard — practical, physical, social and emotional.
2. Beside each, jot one idea for how it might be handled (even a rough one).
3. Mark any barrier that needs medical help (e.g., withdrawal) so your therapist can involve the doctor.

Worked example: Barrier “withdrawal symptoms” possible solution “ask doctor about medicine” (medical). Barrier “friends who use” “avoid the lane, tell them I'm off it.” Barrier “no money for the clinic” “ask about free services.”

What might get in the way (barrier)	One idea for handling it

Clinician review: treat barriers as problems to solve *with* the patient, not reasons change won't work. Route medical barriers (withdrawal, pain) to the treating doctor; carry social barriers into the coping plan.

Worksheet 12 · My Coping Options & Support Network

Purpose: to match a practical coping response and a source of support to each of your main triggers, so you're never caught without a plan. **Best used in:** Sessions 8–9.

How to fill this in:

1. Copy your top triggers from Worksheet 10 into the first column.
2. For each, write a coping option *you* would actually use (distraction, leaving, calling someone, a routine change).
3. Name a specific person or service who can support you with that trigger.
4. Then list your overall support network at the bottom.

Worked example: Trigger “evening boredom” coping “go for a walk / call my brother” support “brother”. Trigger “payday” coping “give the money to my wife the same day” support “wife”.

My trigger	My coping response	Who/what supports me

My overall support network (people, groups, clinic, helpline):

Clinician review: ensure coping options are the patient's own choices and rehearse at least one in session. If the network is thin, build in formal supports (clinic, mutual-help groups, helplines).

Worksheet 13 · My Craving Management Plan

Purpose: to have a clear, practised plan for when a craving hits — because cravings are strong but temporary, and they pass. **Best used in:** Session 8.

How to fill this in:

1. Rate how strong your cravings usually get (0–10) and note what usually sets them off.
2. Learn the two skills in the boxes, then write how you'll use each.
3. Fill the “delay plan”: what you'll do in the first 10–15 minutes of a craving — the window where urges peak and fall.

Worked example: Usual strength 8/10, set off by evening boredom. Urge-surfing: “I'll sit, breathe, and watch the craving like a wave for 10 minutes.” Play-the-tape: “I'll picture the debt and the argument that always follows.” Delay plan: “walk to the shop, call my brother, drink water.”

How strong my cravings usually get (0–10) & what sets them off:

Urge-surfing — ride the craving out like a wave; it rises, peaks and falls without you acting.
My urge-surfing plan:

Play the tape forward — before using, picture the *whole* outcome, not just the first high.
My “tape”:

My delay plan — what I'll do in the first 10–15 minutes of a craving:

Clinician review: rehearse urge-surfing in session; make the delay plan concrete and portable. Reinforce that acting on the plan — not the absence of cravings — is success.

Worksheet 14 · My Change Plan

Purpose: to put your decision into a clear plan — in your own words. This is *your* plan, not one handed to you, which is exactly why it works. **Best used in:** Session 7 (once you feel ready).

How to fill this in:

1. Write the change you want in your own words — it can be stopping completely or cutting down.
2. Fill every line; keep the steps small and realistic.
3. Name real people who'll help and exactly how.
4. Finish the obstacle line — knowing what could go wrong is how you stay ready.

Worked example: Change: “stop smack, halve my cigarettes.” Reasons: “my daughter, my health, money.” Steps: “see the doctor about medicine; avoid the market lane; tell my wife.” Help: “wife holds the money; brother checks in daily.” I'll know it's working when: “I sleep better and start saving.” Obstacles & response: “cravings and payday call my brother, keep busy.”

The change I want to make:

My most important reasons for this change:

The specific steps I'll take:

How others can help, and who:

I'll know the plan is working when:

Things that could get in the way, and how I'll handle them:

Clinician review: read the plan back and affirm commitment language. The patient keeps a copy. If the plan sounds like the therapist's rather than the patient's, re-elicit it in their words.

Worksheet 15 · My First Steps (Action)

Purpose: to turn your change plan into small, concrete actions you can start *this week* — early wins that build momentum. **Best used in:** end of Session 7.

How to fill this in:

1. Choose 2–4 actions you can realistically start in the next 7 days.
2. For each, write exactly *when* you'll do it and *who/what* will help.
3. Keep them small enough that you're fairly sure you'll succeed — success builds confidence.

Worked example: 1) “Book the doctor's appointment — tomorrow morning — wife reminds me.” 2) “Tell my wife the plan — tonight.” 3) “Avoid the market lane after work — every day this week — take the longer route.”

Step I'll take this week	When	Who/what helps

How I'll reward myself for completing a step:

Clinician review: keep steps small, specific and time-bound; agree a check-in. If a step is over-ambitious, scale it down to guarantee an early success.

Worksheet 16 · Family / Support-Person Involvement

Purpose: to plan how the people around you can genuinely help — *supporting* you, not policing you. **Best used in:** Session 9, with your consent (and the support person present if you wish).

How to fill this in:

1. Name your key support people.
2. Write the specific, *helpful* things they can do.
3. Write the things that are *not* helpful, so everyone is clear.
4. Note what you'd like them to know about your plan.

Worked example: Helpful: “hold the money, celebrate my clean days, drive me to the clinic.” Not helpful: “checking my pockets, bringing up old failures during arguments.”

My key support people:

Specific helpful things they can do:

Things that are NOT helpful (to avoid):

What I'd like them to know about my plan:

Clinician review: protect the patient's autonomy and confidentiality; avoid turning supports into surveillance. Consider a joint session. If the family is conflictual, keep the patient's goals central and refer for family work if needed.

Worksheet 17 · My Progress Review

Purpose: to look honestly at how things have gone — celebrating what worked and learning from what didn't. **Best used in:** Session 10 and at follow-ups.

How to fill this in:

1. Think back over the time since your last session.
2. Record your wins first — including small ones.
3. Note anything hard or any slips, without judging yourself.
4. Decide one thing to adjust going forward.

Worked example: Went well: “18 of 21 days without smack; cigarettes down to 10/day; sleeping better.” Hard/slips: “one slip on payday.” Adjust: “give my wife the money on payday itself.”

What went well since last time:

What was hard, and any slips (no blame — just facts):

One thing I want to adjust going forward:

Clinician review: affirm effort and attribute success to the patient. Frame slips as learning (route to Worksheet 18). If discouraged, point to concrete evidence of progress to rebuild self-efficacy.

Worksheet 18 · Learning from a Lapse

Purpose: to understand a slip without shame, and turn it into useful learning — so one lapse doesn't become a full relapse. **Best used in:** after any lapse.

How to fill this in:

1. Remember: a lapse is a single slip; it does not erase your progress or mean failure.
2. Describe what led up to it — the trigger and situation.
3. Spot the early warning signs you can watch for next time.
4. Note what you'll do differently — and importantly, what you did *well* (like stopping or reaching out).

Worked example: Trigger: “payday and I bumped into an old friend.” Warning sign missed: “I was carrying extra cash.” Differently: “give the money to my wife; avoid that route on payday.” Did well: “I stopped after one and called my brother.”

What led to the lapse (trigger & situation):

Early warning signs I can watch for next time:

What I'll do differently:

What I did well / how I recovered:

Clinician review: normalise lapses as common, not failure; separate lapse from relapse; de-shame. Update the change & relapse-prevention plans with what was learned.

Worksheet 19 · Relapse-Prevention & Emergency Plan

Purpose: to prepare clear “if-this-then-that” responses for high-risk moments, plus an emergency plan you can act on instantly. **Best used in:** Session 11. Keep a copy on your phone or in your wallet.

How to fill this in:

1. In each row, write a likely high-risk situation or warning sign, then exactly what you'll do (your “if-then” plan).
2. Fill in your emergency contacts.
3. Complete the crisis/overdose steps — vital if you use opioids.

Worked example: If “offered at a function” then “say I'm off it and step outside to call my brother.” If “skipping clinic appointments” then “tell my wife and rebook immediately.” Opioid safety: “keep naloxone at home; never use alone.”

If this happens (high-risk situation / warning sign)	Then I will… (my response)

My emergency contacts (name & number):

Crisis / overdose steps (opioids: naloxone location; call emergency services; never use alone):

Clinician review: rehearse at least one scenario in session; keep the plan concrete and portable. For opioid users, confirm naloxone access and overdose-prevention knowledge.

Worksheet 20 · My Treatment Journey Review

Purpose: to look back over the whole journey, recognise how far you've come, and carry a strong sense of "who I am now" into the future. **Best used in:** Session 12.

How to fill this in:

1. Think back to how you felt in the very first session.
2. Name the biggest changes you've made.
3. Write what you've learned about yourself.
4. Note what you're proudest of and how you'll protect it.

Worked example: Started: "unsure change was even possible." Biggest changes: "10 weeks clean, trust rebuilt with my daughter, back at work steadily." Learned: "I'm more determined than I thought." Proudest: "my daughter trusts me again — I'll protect it by keeping my routines and support."

Where I started:

The biggest changes I've made:

What I've learned about myself:

What I'm proudest of & how I'll protect it:

Clinician review: reinforce identity/consolidation change talk ("I'm the kind of person who…"). Link to the continuing-care plan (Worksheet 21). If the patient minimises progress, point to concrete evidence from earlier worksheets.

Worksheet 21 · My Continuing-Care Plan

Purpose: to make sure you have ongoing support in place after these sessions end — the stage where many people drift without a plan. **Best used in:** Session 12 / discharge.

How to fill this in:

1. List any ongoing treatment and your next appointments.
2. Name the people and groups who'll support you going forward.
3. Write a clear emergency plan — who you'll call and what you'll do if you're struggling.
4. Set a specific date to review how you're doing.

Worked example: Treatment: “OST clinic monthly.” Support: “NA meeting on Fridays; brother; wife.” Emergency: “if struggling, call my brother, then the clinic helpline; keep naloxone at home.” Review date: “in 4 weeks.”

Ongoing treatment & appointments:

Support people & groups:

Emergency plan (who I call; crisis/overdose steps):

My next review date:

Clinician review: confirm referrals are real and booked; include overdose/crisis steps for opioid users. If access is poor, build in community, helpline and family resources and document the gap.

Worksheet 22 · My SHORT-TERM Goals (MET)

Purpose: to set small, clear, achievable goals for the **next 1–4 weeks** that build momentum and early success toward your bigger aims. In MET, short-term goals turn motivation into movement. **Best used in:** Session 7 onward, alongside the Change Plan (Worksheet 14).

How to fill this in (step by step):

1. Pick **2–4** goals you can realistically reach soon — stepping stones, not the whole mountain.
2. Make each goal **SMART**: **S**pecific (exactly what), **M**easurable (how you'll know), **A**chievable (within reach), **R**elevant (matters to you), **T**ime-bound (by when).
3. For each, write how you'll measure it and the first support/step.
4. Rate your confidence (0–10) for each goal. If a goal is under 7, make it smaller so you're likely to succeed.
5. Remember: aiming to *reduce* or *stabilise* is a valid short-term goal if that's where you are — it doesn't have to be full abstinence yet.

Worked example (opioid/smack user):

Short-term goal	By when	How I'll measure it	Support / first step	Conf.
Attend doctor's appointment for OUD medicine	This week	Appointment attended; prescription started	Wife reminds me; I book tomorrow	8
Cut cigarettes 20 10/day	2 weeks	Daily tally in phone	Nicotine gum; keep count	6 smaller
Avoid the market lane after work	Daily this week	Avoided it today? Y/N	Longer route; call brother at 6pm	7

My short-term goal (next 1–4 weeks)	By when	How I'll measure it	Support / first step	Conf. /10

- ☐ These goals are things I chose, not goals imposed on me.
- ☐ Each goal is small enough that I'm fairly confident I can do it.
- ☐ Each short-term goal connects to one of my long-term goals (Worksheet 23).

Clinician review: confirm goals are the patient's own, specific and scaled for an early win; use the confidence ruler on each — if under 7, shrink the goal. Link each short-term goal to a trigger/coping option (Worksheets 10/12) and to a long-term goal (23). Review attainment at the next contact and affirm effort. **Common difficulties:** goal too big/abstract ("be a better person") break into a concrete behaviour this week; wants immediate abstinence but low confidence honour the aim and add a smaller stepping-stone; therapist-pleasing goal "What would *you* find useful this week?"; goal not met treat as information, scale down, no blame.

Worksheet 23 · My LONG-TERM Goals (MET)

Purpose: to connect your day-to-day efforts to the **bigger life you want (3 months to a year or more)** — the deeper "why" that keeps you going when motivation dips. In MET, long-term goals give short-term goals their meaning. **Best used in:** Sessions 7 & 12 and continuing care; revisit regularly.

How to fill this in (step by step):

1. Picture yourself 3, 6 and 12 months from now. What do you want your *life* to look like — not just your substance use, but relationships, work, health, self-respect?
2. Write each long-term goal, with a rough timeframe.
3. For each, write **why it matters** — the value behind it (link to Worksheet 8). This is the engine of your motivation.
4. List the **short-term steps** (from Worksheet 22) that build toward each long-term goal, so the path is visible.
5. Finish the single sentence at the bottom — your overall vision.

Worked example (alcohol user):

Long-term goal	Timeframe	Why it matters (my value)	Short-term steps toward it
Stay alcohol-free & rebuild trust at home	6–12 months	Being a reliable husband & father	No weekday drinking now full abstinence; weekly check-in with wife
Clear my debts & manage money	12 months	Security & self-respect	Salary into joint account; track spending monthly
Return to weekend cricket	3–6 months	Health & belonging without alcohol	Join the Sunday team; replace the bar with the ground

My long-term goal (3–12+ months)	Timeframe	Why it matters (my value)	Short-term steps toward it

In one sentence, the life I'm working toward is:

- [ ] My long-term goals reflect **my** values and the life **I** want.
- [ ] I can see how my short-term goals (Worksheet 22) connect to these.

Clinician review: use long-term goals to develop discrepancy and to anchor motivation when it dips. Tie each goal to a value (Worksheet 8) and to concrete short-term goals (22) so the route is visible; revisit at follow-ups and boosters. **Common difficulties:** no vision ("can't think past today") start from values and one modest hope; unrealistic/overwhelming affirm the aspiration, focus on the next short-term step; only externally-imposed goals find an overlapping personal benefit to build intrinsic motivation; a setback shakes the goals reframe as a detour, not a dead end.

Worksheet 24 · Clinician MET Session Note

Purpose: concise, non-stigmatising clinical documentation after each MET session. **For:** the clinician. **Used:** after every session.

How to complete: record the patient's own words and your observations — not labels. Write “weighing reasons for and against change; importance 6, confidence 3” rather than “resistant/unmotivated”. Note safety and coordination explicitly.

Session number & date	
Presenting concerns	
Patient's perspective (their words)	
Stage / readiness	
Importance (0–10) & Confidence (0–10)	
Change talk noted	
Sustain talk / ambivalence noted	
Techniques / interventions used	
Worksheets used	
Patient response	
Agreed action / homework	
Safety concerns (OD, suicidality, withdrawal)	
Referral / treatment coordination	
Plan for next session	

Note: document ambivalence descriptively, never as a character judgement. Flag any safety concern for immediate medical/psychiatric action.

Dango_Psych — Tejas Dangodra

M.Phil. Clinical Psychology

MET Book Two · Session-by-Session Guide & Worksheet Pack

Built on the Project MATCH MET Manual (Miller, Zweben, DiClemente & Rychtarik), Miller & Rollnick's *Motivational Interviewing* (3rd ed.), and Naar-King & Suarez's *Motivational Interviewing with Adolescents and Young Adults*. The 12-session structure is a supplementary clinical extension of the brief published model. Educational/clinical-training use — not a substitute for medical care, supervision, or local clinical guidelines. MET complements, and does not replace, medical evaluation, detox, overdose prevention and pharmacotherapy.